



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متدد

إسم المريض ربيعاً : ساميه فليفل جرجس فليفل

العنوان : **/

المحافظة : القاهرة

رقم البطاقة : 26809090101902 رقم التليفون : 01206981962 تاريخ تسجيل الإستمارة : 03-10-2026

العمر (تاريخ الميلاد) : 09-09-1968 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مسح ذري علي الجسم [8530] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: ساميه فليفل جرجس فليفا
National ID: 26809090101902



تقرير طبي

السيد

تحية طبية وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ ومساءة الخروج :
PM 01:17 05-07-2026

تاريخ ومساءة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Whole-body PET/CT scan

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

سامية
فليفل جرجس فليفل
٦٧ ش طارق عابدين
المرج - القاهرة

١٩٦٨/٠٩

٢٦٨.٩.٩.١.١٩.٢

LW5971294

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٢٠٢٦/٠٧ ٢٦٨.٩.٩.١.١٩.٢

بدون عمل

أنثى مسيحية متروجة
سمعان سيد رزيق

البطاقة سارية حتى ٢٠٣٣/٠٧/٢٤

١

Patient's Name: Samia Filfil Girgis Filfil
Date: 2026-08-16
Referred by: Prof. Dr. no doctor
Hospital: Goustave Roussey International Egypt



Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:
Clinical summary: right breast lump and nipple retraction of couple of months duration.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ anterior third irregular speculated high density mass lesion with middle & posterior third focal asymmetry; all with coarse heterogenous calcifications within & surrounding distortion.
- Thickened skin, coarsened trabeculations and retracted nipple.
- **Ultrasound** demonstrated UOQ 10 o'clock, all through the breast thickness segmental pathological soft tissue echogenicity filled prominent ducts reaching 3 mm in caliber with surrounding abnormal vascularity. Two connected masses by the previously mentioned ducts are noted, (**Mass A**) middle third, 6 mm deep to the skin, 11 mm from index measuring 20 x 9 mm & (**index lesion**) anterior third/retro-areolar, 3.5 mm deep to the skin & reaching the nipple, measuring 24 x 17 mm. those two masses were heterogenous hypoechoic anti-parallel and showed internal calcifications and mild internal vascularity.
- Thickened skin reaching 5 mm with coarsened trabecular.
- Retracted nipple.
- Multiple (more than three) pathological axillary LNs are noted at level 1, 2 & 3, one level 1 few matted ones forming an irregular mass 1 measuring 33 x 25 mm.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- Scattered few small circumscribed rounded equal density mass lesions with no speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with few scattered small simple cysts measuring 5-8 mm each, No solid lesions.
- Normal skin/nipple/areola complex.
- Multiple (more than three) pathological axillary LNs are noted at level 1, 2 & 3, largest one at level 1 measuring 22 x 17 mm.

Conclusion:-

Right breast (BIRADS 4a): Pathological axillary nodes.

Left breast (BIRADS 5): Locally advanced highly suspicious UOQ process with ipsilateral pathological level 1, 2 axillary lymph nodes forming nodal mass at former level.

Recommendation:-

For US guided biopsy of right UOQ masses as well as left axillary nodes and histopathological correlation.

Best Regards,
DR. Amal Hegazi, M.Sc.

جمهورية مصر العربية
بطاقة تحليل الشخصية



سامية

فنيكل جرجس فنيكل

٦٧ ش طارق عابدين

المرج - القاهرة



٢٦٨٠٩٠٩٠١٠١٩٠٢

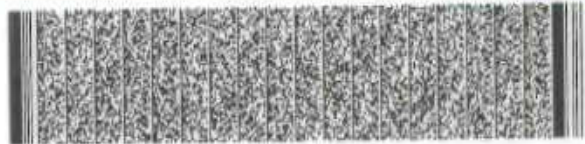
LW5971294

٢٠٢٦/٠٧ ٢٦٨.٩٠٩.١.١٩٠٢
بدون عمل

أنثى مسيحية متزوجة
سمعان سيد رزيق

البطاقة سارية حتى ٢٠٢٣/٠٧/٢٤

١



Patient's Name:

Samia F

Date:

2026-08-16

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: right breast lump and nipple retraction of couple of months duration.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ anterior third irregular speculated high density mass lesion with middle & posterior third focal asymmetry; all with coarse heterogeneous calcifications within & surrounding distortion.
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- Multiple (more than three) pathological axillary LNs are noted at level 1, 2 & 3, largest one at level 1 measuring 22 x 17 mm.

Conclusion:-

- **Left breast (BIRADS 4a): Pathological axillary nodes.**
- **Right breast (BIRADS 5): Locally advanced highly suspicious UOQ process with ipsilateral pathological level 1, 2 axillary lymph nodes forming nodal mass at former level.**

Recommendation:-

- **For US guided biopsy of right UOQ masses as well as left axillary nodes and histopathological correlation.**

Best Regards,

DR. Amal Hegazi, M.Sc.

Name: Mrs. Samia Fifihi Gerjes

Date: 2026-08-16

Bilateral Digital soft tissue mammography with high resolution breast sonography

revealed:

Clinical summary: right breast lump and nipple retraction of couple of months duration.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ anterior third irregular speculated high density mass lesion with middle & posterior third focal asymmetry; all with coarse heterogeneous calcifications within & surrounding distortion.
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Recommendation:-

For US guided biopsy of right UOQ masses as well as left axillary nodes and histopathological correlation.

Best Regards,

DR. Amal Hegazi, M.Sc.

Pathology Report

Clinical Data: Right breast retro-areolar mass (BIRADS 5).

Gross Examination:

- Three grey white tissue cores, the longest is 1.2 cm, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 2).
- Ductal carcinoma in situ: Present, intermediate grade DCIS (Solid and cribriform patterns).
- Microcalcification: Absent.
- Angiolymphatic invasion: Present.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
 - Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).
 - SISH: Not-amplified.
- (Average Her2 copy No.: (2.05), Her2/CEP17 ratio: (2.05))
- Ki67%: 30%.

Patient Name	سامية فيفيل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
Reporting Date	06/9/2026 01:24:03 PM	Pathology No.	2447/26

Conclusion:**Right breast mass, Tru-cut biopsy:**

- Invasive duct carcinoma, Grade 2, with intermediate grade ductal carcinoma in situ (Solid and cribriform pattern).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Equivocal (score 2+).
- SISH: Not-amplified, Group II.
- Ki67%: 30%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

**** ISH groups:**

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).

Patient Name	ساميه فيل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
Reporting Date	06/9/2026 01:24:03 PM	Pathology No.	2447/26

- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).

- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Prof. Dr. Maha Guinei.

Dr. Walaa Ibrahim Ghonain, MD.

MRS SAMIA FELAIFAL

Date : 13/ 7 /2026

Dear Colleague Prof.Dr. MOHAMED MAHER

RIGHT BREAST MAMMOGRAPHY WITH COMPLEMENTARY
ULTRASONOGRAPHY :

- ☐ The RIGHT breast show Tow irregular Partially ill defined dense mass lesions along the retroareolar area and axillary tail
- ☐ No micro nor microcalcification seen
- ☐ Associated thickened edematous skin seen
- ☐ No pathological lymph nodes seen

By complementary U/S

- The right breast show three irregular outline solid masses on the retroareolar area measuring 23 x17 mm and along axillary tail measuring 24x22 mm and in axilla measuring 26 x23 mm with picture of malignant breast masses BIRAD V
- Another note is made of thickened edematous periareolar skin
- One pathological axillary lymph node seen in the axilla measuring 13x15 mm

OPINION :

Picture of right side breast malignant masses

MUCH OBLIGED, YOURS

DR.EID RAMADAN



Patient Name	سامية فيل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
Reporting Date	06/9/2026 01:24:03 PM	Pathology No.	2447/26

Pathology Report

Clinical Data: Right breast retro-areolar mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/08/2026, at 02:00 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Three grey, white tissue cores, the longest is 1.2 cm, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 2).
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- Angioinvasion: Present.
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- Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not-amplified.
- (Average Her2 copy No.: (2.05), Her2/CEP17 ratio: (2.05))
- Ki67%: 30%.

Patient Name	ساميه فيفيل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
Reporting Date	06/9/2026 01:24:03 PM	Pathology No.	2447/26

Conclusion:**Right breast mass, Tru-cut biopsy:**

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Reporting Date	06/9/2026 01:24:03 PM	Pathology No.	2447/26

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- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).

- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Prof. Dr. Maha Guinei.

Dr. Walaa Ibrahim Ghonaim, MD.

MRS SAMIA FELAIFAL

Date : 13/ 7 /2026

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Picture of right side breast malignant masses

MUCH OBLIGED, YOURS

DR.EID RAMADAN



جمهورية مصر العربية

بطاقة تحليل الشخصية



سامية

فنيكل جرجس فنيكل

٦٧ ش طارق عابدين

المرج - القاهرة



٢٦٨٠٩٠٩٠١٠١٩٠٢

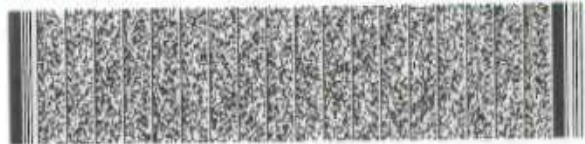
LW5971294

٢٠٢٦/٠٧ ٢٦٨.٩٠٩.١.١٩.٠٢
بدون عمل

أنثى مسيحية متزوجة
سمعان سيد رزيق

٢٠٢٣/٠٧/٢٤ البطاقة سارية حتى

١



Patient's Name:

Samia F

Date:

2026-08-16

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

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Conclusion:-

- **Left breast (BIRADS 4a): Pathological axillary nodes.**
- **Right breast (BIRADS 5): Locally advanced highly suspicious UOQ process with ipsilateral pathological level 1, 2 axillary lymph nodes forming nodal mass at former level.**

Recommendation:-

- **For US guided biopsy of right UOQ masses as well as left axillary nodes and histopathological correlation.**

Best Regards,

DR. Amal Hegazi, M.Sc.

Name: Mrs. Samia Fifihi Gerjes

Date: 2026-08-16

Bilateral Digital soft tissue mammography with high resolution breast sonography

revealed:

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- Scattered fibroglandular densities. (ACR b)
- Scattered few small circumscribed rounded equal density mass lesions with no speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with few scattered small simple cysts measuring 5-8 mm each, No solid lesions.
- Normal skin/nipple/areola complex.
- Multiple (more than three) pathological axillary LNs are noted at level 1, 2 & 3, largest one at level 1 measuring 22 x 17 mm.

Conclusion:-

Left breast (BIRADS 4a): Pathological axillary nodes.

Right breast (BIRADS 5): Locally advanced highly suspicious UOQ process with ipsilateral pathological level 1, 2 axillary lymph nodes forming nodal mass at former level.

Recommendation:-

For US guided biopsy of right UOQ masses as well as left axillary nodes and histopathological correlation.

Best Regards,

DR. Amal Hegazi, M.Sc.

Patient Name	سامية فيل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
Reporting Date	06/9/2026 01:24:03 PM	Pathology No.	2447/26

Pathology Report

Clinical Data: Right breast retro-areolar mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/08/2026, at 02:00 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Three grey, white tissue cores, the longest is 1.2 cm, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 2).
- Ductal carcinoma in situ: Present, intermediate grade DCIS (Solid and cribriform patterns).
- Microcalcification: Absent.
- Angioinvasion: Present.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not-amplified.
- (Average Her2 copy No.: (2.05), Her2/CEP17 ratio: (2.05))
- Ki67%: 30%.

Patient Name	سامية فيفيل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
Reporting Date	06/9/2026 01:24:03 PM	Pathology No.	2447/26

Conclusion:**Right breast mass, Tru-cut biopsy:**

- Invasive duct carcinoma, Grade 2, with intermediate grade ductal carcinoma in situ (Solid and cribriform pattern).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Equivocal (score 2+).
- SISH: Not-amplified, Group II.
- Ki67%: 30%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

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Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

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The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
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Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
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- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Prof. Dr. Maha Guinei.

Dr. Walaa Ibrahim Ghonaim, MD.

MRS SAMIA FELAIFAL

Date : 13/ 7 /2026

Dear Colleague Prof.Dr. MOHAMED MAHER

RIGHT BREAST MAMMOGRAPHY WITH COMPLEMENTARY
ULTRASONOGRAPHY :

- ☐ The RIGHT breast show Tow irregular Partially ill defined dense mass lesions along the retroareolar area and axillary tail
- ☐ No micro nor microcalcification seen
- ☐ Associated thickened edematous skin seen
- ☐ No pathological lymph nodes seen

By complementary U/S

- The right breast show three irregular outline solid masses on the retroareolar area measuring 23 x17 mm and along axillary tail measuring 24x22 mm and in axilla measuring 26 x23 mm with picture of malignant breast masses BIRAD V
- Another note is made of thickened edematous periareolar skin
- One pathological axillary lymph node seen in the axilla measuring 13x15 mm

OPINION :

Picture of right side breast malignant masses

MUCH OBLIGED, YOURS

DR.EID RAMADAN



Patient Name	سامية فيل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
Reporting Date	06/9/2026 01:24:03 PM	Pathology No.	2447/26

Pathology Report

Clinical Data: Right breast retro-areolar mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/08/2026, at 02:00 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Three grey, white tissue cores, the longest is 1.2 cm, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

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Reporting Date	06/9/2026 01:24:03 PM	Pathology No.	2447/26

Conclusion:**Right breast mass, Tru-cut biopsy:**

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IPT:

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Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
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- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Prof. Dr. Maha Guinei.

Dr. Walaa Ibrahim Ghonaim, MD.

MRS SAMIA FELAIFAL

Date : 13/ 7 /2026

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- One pathological axillary lymph node seen in the axilla measuring 13x15 mm

OPINION :

Picture of right side breast malignant masses

MUCH OBLIGED, YOURS

DR.EID RAMADAN



جمهورية مصر العربية

بطاقة تحليل الشخصية



سامية

فنيكل جرجس فنيكل

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المرج - القاهرة



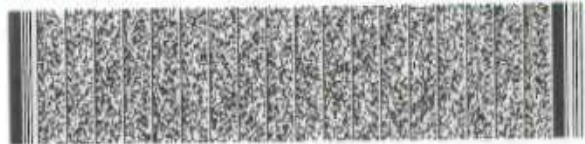
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سمعان سيد رزيق

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البطاقة سارية حتى ٢٠٢٣/٠٧/٢٤



Patient's Name:

Samia F

fil

Date:

2026-08-16

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast

sonography revealed:

Clinical summary: right breast lump and nipple retraction of couple of months duration.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- UOQ anterior third irregular speculated high density mass lesion with middle & posterior third focal asymmetry; all with coarse heterogeneous calcifications within & surrounding distortion.
- Thickened skin, coarsened trabeculations and retracted nipple.
- **Ultrasound** demonstrated UOQ 10 o'clock, all through the breast thickness segmental pathological soft tissue echogenicity filled prominent ducts reaching 3 mm in caliber with surrounding abnormal vascularity. Two connected masses by the previously mentioned ducts are noted, (**Mass A**) middle third, 6 mm deep to the skin, 11 mm from index measuring 20 x 9 mm & (**index lesion**) anterior third/retro-areolar , 3.5 mm deep to the skin & reaching the nipple, measuring 24 x 17 mm. those two masses were heterogeneous hypoechoic anti-parallel and showed internal calcifications and mild internal vascularity.
- Thickened skin reaching 5 mm with coarsened trabecular.
- Retracted nipple.
- Multiple (more than three) pathological axillary LNs are noted at level 1, 2 & 3, one level 1 few matted ones forming an irregular mass 1 measuring 33 x 25 mm.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- Scattered few small circumscribed rounded equal density mass lesions with no speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast parenchyma with few scattered small simple cysts measuring 5-8 mm each, No solid lesions.
- Normal skin/nipple/areola complex.
- Multiple (more than three) pathological axillary LNs are noted at level 1, 2 & 3, largest one at level 1 measuring 22 x 17 mm.

Conclusion:-

- **Left breast (BIRADS 4a): Pathological axillary nodes.**
- **Right breast (BIRADS 5): Locally advanced highly suspicious UOQ process with ipsilateral pathological level 1, 2 axillary lymph nodes forming nodal mass at former level.**

Recommendation:-

- **For US guided biopsy of right UOQ masses as well as left axillary nodes and histopathological correlation.**

Best Regards,

DR. Amal Hegazi, M.Sc.

Name: Mrs. Samia Fifihi Gerjes

Date: 2026-08-16

Bilateral Digital soft tissue mammography with high resolution breast sonography

revealed:

Clinical summary: right breast lump and nipple retraction of couple of months duration.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
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Conclusion:-

Left breast (BIRADS 4a): Pathological axillary nodes.

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Recommendation:-

For US guided biopsy of right UOQ masses as well as left axillary nodes and histopathological correlation.

Best Regards,

DR. Amal Hegazi, M.Sc.

Pathology Report

Clinical Data: Right breast retro-areolar mass (BIRADS 5).

Gross Examination:

- Three grey white tissue cores, the longest is 1.2 cm, totally submitted.

Technique:

- Microscopic examination:**

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 2).
- Ductal carcinoma in situ: Present, intermediate grade DCIS (Solid and cribriform patterns).
- Microcalcification: Absent.
- Angiolymphatic invasion: Present.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
 - Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).
 - SISH: Not-amplified.
- (Average Her2 copy No.: (2.05), Her2/CEP17 ratio: (2.05))
- Ki67%: 30%.

Patient Name	سامية فيفيل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
Reporting Date	06/9/2026 01:24:03 PM	Pathology No.	2447/26

Conclusion:**Right breast mass, Tru-cut biopsy:**

- Invasive duct carcinoma, Grade 2, with intermediate grade ductal carcinoma in situ (Solid and cribriform pattern).

IPT:

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Patient Name	ساميه فيل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
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Signature

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MRS SAMIA FELAIFAL

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- Another note is made of thickened edematous periareolar skin
- One pathological axillary lymph node seen in the axilla measuring 13x15 mm

OPINION :

Picture of right side breast malignant masses

MUCH OBLIGED, YOURS

DR.EID RAMADAN



Patient Name	سامية فيل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
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Pathology Report

Clinical Data: Right breast retro-areolar mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/08/2026, at 02:00 pm. Cold ischemia time is 20 seconds.

Gross Examination:

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Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
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Picture of right side breast malignant masses

MUCH OBLIGED, YOURS

DR.EID RAMADAN



Medical No. : 7553

Patient's Name: Samia Filfil Girgis Filfil

Date: 2026-09-20

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



**MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH
CONTRAST ADMINISTRATION REVEALED:-**

- An enhancing retro-areolar mass lesion is seen in the retro-areolar region of the right breast and measures about 35x23mm(versus 23x17mm in the previous study) with right axillary tail similar mass also seen,measuring about 35x21 mm (versus 24x22mm in dimensions).
- Bilateral axillary lymph nodes are seen,with loss of their hilar lymph nodes , with the largest one on the right side=36x24mm and the largest one on the left side=30x28mm in dimensions.
- Linear atelectatic bands are seen in the middle lobe,lingule and both lower lobes.
- A patchy area of subpleural consolidation is seen in the lingula with few tiny nodules also scattering in the right lung with the largest one seen in the middle lobe and measures about 7.8mm.
- No other sizable abnormally-enhancing pulmonary nodules ,other patchy areas of consolidations or calcifications noted.
- Pretracheal lymph nodes are seen and globally measuring about 27mm.
- No other sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Border line size of the heart with atherosclerotic changes of its big vessels.
- Normal bones of the thoracic cage apart from sternotomy and stitches noted herein.
- Average sized liver,measuring about 14 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 11.8 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal gland, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

Medical No. : 7553

Patient's Name: Samia Filfil Girgis Filfil

Date: 2026-09-20

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



IMPRESSIONS:

A status follow up and comparing with the lastly-done study at 13/07/2026. the current study shows:

- An enhancing mass lesion in the retro-areolar region and another similar lesion in the right axillary tail with increasing size, denoting a progressive course.
- Bilateral axillary and to lesser extent pretracheal lymphadenopathy, with few tiny nodules in the right lung, likely to be metastatic.
- Linear atelectatic bands in the middle lobe, lingula and both lower pulmonary lobes and a patchy area of subpleural consolidation in the lingula.

Prof/Hanan El-Ahwal.

Dr.Hanan Elahwel

Patient Name	سامیه فایز جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
Reporting Date	20/9/2026 09:29:22 AM	Pathology No.	2447/26

Pathology Report

Clinical Data: Right breast retro-areolar mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 13/08/2026, at 02:00 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Three grey, white tissue cores, the longest is 1.2 cm, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 2 (differentiation 3, pleomorphism 2, mitosis 2).
- Ductal carcinoma in situ: Present, intermediate grade DCIS (Solid and cribriform patterns).
- Microcalcification: Absent.
- Angioinvasion: Present.
- Perineural invasion: Absent.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 60% of tumor cells (score 7/8).
- Her2: Moderate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not-amplified.
- (Average Her2 copy No.: (2.05), Her2/CEP17 ratio: (2.05))
- Ki67%: 30%.



Patient Name	سالمية فايفل جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
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Conclusion:**Right breast mass, Tru-cut biopsy:**

- Invasive duct carcinoma, Grade 2, with intermediate grade ductal carcinoma in situ (Solid and cribriform pattern).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 7/8).
- Her2: Equivocal (score 2+).
- SISH: Not-amplified, Group II.
- Ki67%: 30%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

****Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

****ISH groups:**

The ratio is calculated and the results is interpreted as:

- Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.
- Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).



Patient Name	سامية فيول جرجس	Case No.	345864
Sex/Age	Female 57 Y	Patient No.	25508
Registered Date	03/8/2026 03:31:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	31/8/2026 11:47:55 AM	Branch	Gustave Roussy
Reporting Date	20/9/2026 09:29:22 AM	Pathology No.	2447/26

- Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
- Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).

- Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature

Prof. Dr. Maha Guinei.

Dr. Walaa Ibrahim Ghonaim, MD.

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Patient Name	سالمه خليل جرجس	Case No.	360305
Sex/Age	Female 58 Y	Patient No.	26305
Registered Date	24/8/2026 03:39:51 PM	Referred By Dr.	Interventional Radiology
Validation Date	13/9/2026 09:03:41 AM	Branch	Gustave Roussy
Reporting Date	20/9/2026 09:28:53 AM	Pathology No.	2730/26

Pathology Report

Clinical Data: Left axillary lymph node.

Nature of Specimen: Tru-cut biopsy, collected on 24/08/2026, at 3:39 pm, Cold ischemia time: 20 seconds.

Gross Examination: Fragmented white tissue cores, ranging in length 0.5-1.0 cm, totally submitted.

Microscopic Examination:

-Histopathological examination reveals lymphoid cores with preserved lymphoid architecture and reactive follicular hyperplasia. The lymphoid cells are cytologically bland, with no significant atypia or abnormal lymphoid proliferation. No evidence of malignancy is identified in the sections examined.

Diagnosis:

-Left breast mass, Tru-cut biopsy:

- Consistent with lymphoid cores with reactive hyperplasia.
- No malignancy.

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



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